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Haemoglobin

Investigation	Observed Value	Unit	Biological Reference Interval
Haemoglobin (Hb)	**11.1**	g/dL	12.5-16
HCT(Hematocrit)	**34.8**	%	37-47

Dr. Kush Kumar Singh
M.D (Pathology)
(DMC Reg. No. - 39780)

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Haemogram

Investigation	Observed Value	Unit	Biological Reference Interval
Erythrocytes			
Erythrocyte (RBC) Count	**3.21**	mill/cu.mm	4.2-5.4
Haemoglobin (Hb)	**11.1**	g/dL	12.5-16
HCT(Hematocrit)	**34.8**	%	37-47
MCV (Mean Corpuscular Volume)	**108.4**	fL	78-100
MCH (Mean Corpuscular Hb)	**34.4**	pg	27-31
MCHC (Mean Corpuscular Hb Concn.)	**31.8**	g/dL	32-36
RDW (Red Cell Distribution Width)	**16.6**	CV%	11.5-14.0
Leucocytes			
Total Leucocytes (WBC) count	7700	cells/cu.mm	4300-10300
Absolute Neutrophils Count	4158	/c.mm	2000-7000
Absolute Lymphocyte Count	2926	/c.mm	1000-3000
Absolute Monocyte Count	462	/c.mm	200-1000
Absolute Eosinophils Count	154	/c.mm	20-500
Absolute Basophils Count	**0**	/c.mm	20-100
Neutrophils	54	%	40-80
Lymphocytes	38	%	20-40
Monocytes	6	%	2.0-10
Eosinophils	2	%	1-6
Basophils	0	%	0-2
Platelets			
Platelet count	221	10^3 / µl	150-450
MPV (Mean Platelet Volume)	9.0	fL	6-9.5
PCT (Platelet Haematocrit)	0.20	%	0.2-0.5
PDW (Platelet Distribution Width)	**18.3**	%	9-17

EDTA Whole Blood : Test is done on Automated Five Part Cell Counter. Hemoglobin is measured by Spectrophotometry method. WBC, RBC and Platelet Count are measured by Coulter Principle (Impedance Method). WBC Differential is done by VCS Method. MCV and RDW are derived from RBC histogram. MPV and PDW are derived from Platelet histogram. Calculated Parameters are: HCT, MCH, MCHC, PCT and Absolute WBC counts. All abnormal hemogram are reviewed and confirmed microscopically. Differential count is based on approximately 10,000 cells.

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HbA1c- Glycated Haemoglobin, blood by HPLC method

(EDTA Whole Blood)

Investigation	Observed Value	Unit	Biological Reference Interval
HbA1C- Glycated Haemoglobin (HPLC)	5.6	%	Non-diabetic: <= 5.6 Pre-diabetic: 5.7-6.4 Diabetic: >= 6.5
Estimated Average Glucose (eAG) (Calculated)	114.02	mg/dL	

Interpretation & Remark:

1. HbA1c is used for monitoring diabetic control. It reflects the estimated average glucose (eAG).
2. HbA1c has been endorsed by clinical groups & ADA (American Diabetes Association) guidelines 2022, for diagnosis of diabetes using a cut-off point of 6.5%.
3. Trends in HbA1c are a better indicator of diabetic control than a solitary test.
4. Low glycated haemoglobin(below 4%) in a non-diabetic individual are often associated with systemic inflammatory diseases, chronic anaemia(especially severe iron deficiency & haemolytic), chronic renal failure and liver diseases. Clinical correlation suggested.
5. To estimate the eAG from the HbA1C value, the following equation is used: eAG(mg/dl) = 28.7*A1c-46.7
6. Interference of Haemoglobinopathies in HbA1c estimation.
   A. For HbF > 25%, an alternate platform (Fructosamine) is recommended for testing of HbA1c.
   B. Homozygous hemoglobinopathy is detected, fructosamine is recommended for monitoring diabetic status
   C. Heterozygous state detected (D10/ turbo is corrected for HbS and HbC trait).
7. In known diabetic patients, following values can be considered as a tool for monitoring the glycemic control. Excellent Control - 6 to 7 %, Fair to Good Control - 7 to 8 %, Unsatisfactory Control - 8 to 10 % and Poor Control - More than 10 % .

Note : Hemoglobin electrophoresis (HPLC method) is recommended for detecting hemoglobinopathy.

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Investigation	Observed Value	Unit	Biological Reference Interval
TOTAL HEALTH ESSENTIALS			
Glucose fasting (Plasma-F,Hexokinase)	83.4	mg/dL	Normal: 70-99 Impaired Tolerance: 100-125 Diabetes mellitus: >= 126 (on more than one occasion) (American diabetes association guidelines 2021)

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Investigation	Observed Value	Unit	Biological Reference Interval
Lipid Profile-Mini			
Cholesterol (Total) (Serum,Cholesterol Oxidase- Peroxidase)	168.5	mg/dL	Desirable: < 200 Borderline High: 200-239 High: >= 240
Triglycerides level (Serum,Glycerol Phosphate Oxidase)	108.0	mg/dL	Normal: < 150 Borderline High: 150-199 High: 200-499 Very High: >= 500
HDL Cholesterol (Serum,Accelerator Selective Detergent)	47.4	mg/dL	Major risk factor for heart disease: <= 40 Negative risk factor for heart disease: >= 60
Non HDL Cholesterol (Serum,Calculated)	121.1	mg/dL	Optimal: < 130 Desirable: 130-159 Borderline high: 159-189 High: 189-220 Very High: >= 220
LDL Cholesterol (Serum,Calculated)	99.50	mg/dL	Optimal: < 100 Near Optimal: 100-129 Borderline high: 130-159 High: 160-189 Very High: >= 190
VLDL Cholesterol (Serum,Calculated)	21.6	mg/dL	< 30
LDL/HDL RATIO (Serum,Calculated)	**2.1**		2.5-3.5
CHOL/HDL RATIO (Serum,Calculated)	3.55		3.5-5

Note: Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

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Investigation	Observed Value	Unit	Biological Reference Interval
25 Hydroxy (OH) Vit D (Serum,ECLIA)	**5.18**	ng/mL	Deficiency: < 10 Insufficiency: 10-30 Sufficiency: 30-100 Hypervitaminosis: > 100

Interpretation :

1. Vitamin D is a fat soluble vitamin and exists in two main forms as cholecalciferol(vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight exposure & Ergocalciferol(vitamin D2) present mainly in dietary sources.Both cholecalciferol & Ergocalciferol are converted to 25(OH)vitamin D in liver.
2. Testing for 25(OH)vitamin D is recommended as it is the best indicator of vitamin D nutritional status as obtained from sunlight exposure & dietary intake. For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D, serum calcium, serum PTH & serum alkaline phosphatase.
3. During monitoring of oral vitamin D therapy- suggested testing of serum 25(OH)vitamin D is after 12 weeks or 3 mths of treatment. However, the required dosage of vitamin D supplements & time to achieve sufficient vitamin D levels show significant seasonal(especially winter) & individual variability depending on age, body fat, sun exposure, physical activity ,genetic factors(especially variable vitamin D receptor responses), associated liver or renal disease, malabsorption syndromes and calcium or magnesium deficiency influencing the vitamin D metabolism Vitamin D toxicity is known but very rare.kindly correlate clinically, repeat with fresh sample if indicated.

Abbreviation :
ECLIA : Electrochemiluminescence Immunoassay

Associated Test Profile :

- For diagnosis of vitamin D deficiency it is recommended to have clinical correlation with serum 25(OH)vitamin D and serum PTH.An inverse relationship exists between PTH and 25(OH)D levels, Parathyroid hormone levels start to rise at 25(OH)D levels below 31 ng/mL & usually decrease after the correction of vitamin D insufficiency.Thus, restoration of PTH and 25 (OH)D levels to normalcy after adequate vitamin D replacement therapy is a useful monitoring strategy.
- As a holistic & scientific approach for diagnosis and optimal treatment for vitamin D deficiency, Vitamin D plus profile (25 Hydroxy(OH) Vit D and PTH) is suggested.

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Investigation	Observed Value	Unit	Biological Reference Interval
Vitamin B12 level (Serum,ECLIA)	**Below 100**	pg/mL	197-771

Interpretation :

1. Vit B12 levels are decreased in megaloblastic anemia, partial/total gastrectomy, pernicious anemia, peripheral neuropathies, chronic alcoholism, senile dementia, and treated epilepsy.
2. An associated increase in homocysteine levels is an independent risk marker for cardiovascular disease and deep vein thrombosis.
3. HoloTranscobalamin II levels are a more accurate marker of active VitB12 component.

Abbreviation :
ECLIA : Electrochemiluminescence Immunoassay

Calcium (Serum,Arsenazo III dye)	8.88	mg/dL	8.4-10.2
Electrolytes (Serum,Ion selective electrode, indirect)			
Sodium	143	mEq/L	136-145
Potassium	4.52	mEq/L	3.5-5.1
Chlorides	106.0	mEq/L	98-107
Uric Acid (Serum,Uricase)	5.3	mg/dL	2.6-6
Alkaline Phosphatase (Serum,para-Nitrophenyl-phosphate)	103	U/L	35-104
SGOT (AST) (Serum,NADH without P5P)	16.8	U/L	0-31
SGPT (ALT) (Serum,NADH without P5P)	9.9	U/L	0-34
BilirubinTotal, Direct, IndirectSerum			
Bilirubin-Total (Serum,Diazotized Sulfanilic Acid (Modified Jendrassik & Grof))	0.42	mg/dL	0.2-1.2
Bilirubin-Direct (Serum,Diazotized Sulfanilic Acid (Modified Jendrassik & Grof))	0.19	mg/dL	0.0-0.5
Bilirubin- Indirect (Serum,Calculated)	0.23	mg/dL	0.1-1.0
Gamma GT (GGTP) (Serum,L-Gamma-glutamyl-3-carboxy -4 - nitroanalyte substrate, IFCC)	15	U/L	9-36

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Investigation	Observed Value	Unit	Biological Reference Interval
Proteins			
Total Protein (Serum,Biuret)	7.43	g/dL	6.2-8.1
Albumin (Serum,Bromocresol green)	4.15	g/dL	3.2-4.6
Globulin (Serum,Calculated)	3.28	g/dL	1.8-3.6
A/G Ratio (Serum,Calculated)	1.27		1.1-2.2
Testosterone (Total) (Serum,CMIA)	26.79	ng/dL	12-36

Abbreviation :
CMIA : Chemiluminescence Microparticle Immunoassay

E2 Estradiol (Serum,ECLIA)			
E2 - Estradiol	34.5	pg/mL	Menstrual Cycle : Follicular phase: 21-251 Midcycle peak: 38-649 Luteal phase: 21-312 Post Menopausal Females : not on HRT : <= 28 on HRT: <= 144

Abbreviation :
ECLIA : Electrochemiluminescence Immunoassay

CRP - C Reactive Protein (Serum,Immunoturbidimetric Assay)	**8.24**	mg/L	< 5.0

Interpretation :

1. Measurement of CRP is useful for the detection and evaluation of infection, tissue injury, inflammatory disorders and associated diseases .
2. High sensitivity CRP (hsCRP) measurements may be used as an independent risk marker for the identification of individual at risk for future cardiovascular disease.
3. Increase in CRP values are non-Specific and should not be interpreted without a complete history.

Note :
If the test has been ordered for COVID-19 purpose, you may take one of the following profiles for further investigation under your clinician's advice.

1. Covid Monitor Initial profile (C0374) from Day 1 to Day 5
2. Covid Monitor maintenance profile (C0375) from Day 5 to Day 10
3. Covid Monitor Recovery profile (C0376) after discharge.

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Investigation	Observed Value	Unit	Biological Reference Interval
Insulin (Fasting) (Serum,ECLIA)	6.31	µIU/mL	Fasting: 2-25

Interpretation :

1. Levels are increased in insulinomas, factitious hypoglycemia, insulin autoimmmune syndrome, acromegaly (after ingestion of glucose), Cushings syndrome, corticosteroid administration and levodopa usage.
2. Levels are depressed to absent in diabetes mellitus, pituitary tumors and chronic pancreatic diseases i.e. cystic fibrosis.
3. Insulin/ C-peptide ratio is used for differentiating between factitious hypoglycemia and insulinomas where a ratio< 1.0 indicates insulinoma; but results may vary in renal failure.
4. Antibodies to insulin form in longstanding diabetes mellitus treated with insulin hence in these patients monitoring insulin levels gives better prognosis.

Abbreviation :
ECLIA : Electrochemiluminescence Immunoassay

BUN-Creatine Ratio

Creatinine (Serum,Modified Jaffe)	**0.92**	mg/dL	0.50-0.90
BUN-Blood Urea Nitrogen (Serum,Urease)	15.2	mg/dL	8-23

Remark: In blood, Urea is usually reported as BUN and expressed in mg/dl. BUN mass units can be converted to urea mass units by multiplying by 2.14.

Urea Serum (Serum)	32.53	mg/dL	21-43
BUN / CREAT RATIO (Serum)	16.52		
SODIUM/POTASSIUM RATIO (Serum)	31.63		
TRIGLYCERIDE/HDL RATIO (Serum,Calculated)	2.27		
Phosphorous (Serum,Phospomolybdate)	4.30	mg/dL	2.3-4.7
Free T3 (Serum,ECLIA)	2.97	pg/mL	2.0-4.4 First Trimester :2.46 - 3.49 Second Trimester : 2.09 - 3.55 Third trimester : 2.01 - 3.27

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Investigation	Observed Value	Unit	Biological Reference Interval

Interpretation :

The free fraction represents the physiologically active thyroid hormone. Total T3 & T4 values may also be altered in other conditions due to changes in serum proteins or binding sites Pregnancy, Drugs (Androgens,Estrogens, O C Pills ,Phenytoin), Nephrosis etc. In such cases Free T3 and Free T4 give corrected values.

Abbreviation :
ECLIA : Electrochemiluminescence Immunoassay

Free T4 (Serum,ECLIA)	0.94	ng/dL	0.93-1.7 First Trimester : 0.7-2.0 Second Trimester : 0.5-1.6 Third Trimester : 0.5-1.6

Interpretation :

It represents physiologically active thyroid hormone. Total T3 & T4 values may also be altered in other conditions due to changes in serum proteins or binding sites Pregnancy, Drugs (Androgens,Estrogens, O C Pills ,Phenytoin), Nephrosis etc. In such cases Free T3 and Free T4 give corrected values.

Abbreviation :
ECLIA : Electrochemiluminescence Immunoassay

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Investigation	Observed Value	Unit	Biological Reference Interval
TOTAL HEALTH ESSENTIALS			
Thyroid panel-1 (Serum,ECLIA)			
T3 (Total)	117	ng/dL	84.6-201.8 Second Trimester : 128.9 - 262.3 First Trimester : 104.8 - 229.8 Third trimesters : 135.4 - 261.7
T4 (Total)	6.84	µg/dL	5.1-14.1 First Trimester : 7.33 - 14.8 Second Trimester : 7.93 - 16.1 Third Trimester : 6.95 - 15.7
TSH(Ultrasensitive)	**10.100**	µIU/mL	0.54-5.3 First Trimester : 0.33-4.59 Second Trimester : 0.35-4.10 Third trimester : 0.21-3.15

INTERPRETATION

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TSH	T3 / FT3	T4 / FT4	Suggested Interpretation for the Thyroid Function Tests Pattern
Within Range	Decreased	Within Range	• Isolated Low T3-often seen in elderly & associated Non-Thyroidal illness. In elderly the drop in T3 level can be upto 25%.
Raised	Within Range	Within Range	•Isolated High TSHespecially in the range of 4.7 to 15 mIU/ml is commonly associated with Physiological & Biological TSH Variability. •Subclinical Autoimmune Hypothyroidism •Intermittent T4 therapy for hypothyroidism •Recovery phase after Non-Thyroidal illness"
Raised	Decreased	Decreased	•Chronic Autoimmune Thyroiditis •Post thyroidectomy,Post radioiodine •Hypothyroid phase of transient thyroiditis"
Raised or within Range	Raised	Raised or within Range	•Interfering antibodies to thyroid hormones (anti-TPO antibodies) •Intermittent T4 therapy or T4 overdose •Drug interference- Amiodarone, Heparin,Beta blockers,steroids, anti-epileptics"
Decreased	Raised or within Range	Raised or within Range	•Isolated Low TSH -especially in the range of 0.1 to 0.4 often seen in elderly & associated with Non-Thyroidal illness •Subclinical Hyperthyroidism •Thyroxine ingestion"
Decreased	Decreased	Decreased	•Central Hypothyroidism •Non-Thyroidal illness •Recent treatment for Hyperthyroidism (TSH remains suppressed)"
Decreased	Raised	Raised	•Primary Hyperthyroidism (Graves' disease),Multinodular goitre, Toxic nodule •Transient thyroiditis:Postpartum, Silent (lymphocytic), Postviral (granulomatous,subacute, DeQuervain's),Gestational thyrotoxicosis with hyperemesis gravidarum"
Decreased or within Range	Raised	Within Range	•T3 toxicosis •Non-Thyroidal illness

References: 1. Interpretation of thyroid function tests. Dayan et al. THE LANCET • Vol 357 • February 24, 2001
2. Laboratory Evaluation of Thyroid Function, Indian Thyroid Guidelines, JAPI, January 2011,vol. 59

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Investigation	Observed Value	Unit	Biological Reference Interval
TOTAL HEALTH ESSENTIALS			
Iron Studies, Serum (Serum)			
Iron (FerroZine)	55.1	µg/dL	33-193
TIBC (Total Iron BInding Capacity) (Calculated)	335	µg/dL	250-450
UIBC (Unsaturated Iron Binding Capacity) (FerroZine)	279.9	µg/dL	135-392
Transferin Saturation (Calculated)	16	%	14-50

Interpretation :

1. Measurements of serum iron, TIBC and the percentage of iron saturation of transferrin are useful screening tests for iron deficiency anaemia.
2. However, serum iron exhibits significant diurnal variation and may transiently rise or reach reference values after dietary or iron supplements & post blood transfusion.
3. The diagnostic specificity of a low serum iron for iron deficiency is lost in the presence of acute & chronic inflammatory processes as the concentrations of iron and transferrin in the serum are significantly affected, and fall rapidly as part of the acute phase response irrespective of the iron stores status in the body.
4. Hence, Concurrent measurement of the markers mentioned in the below interpretative table alongwith serum iron studies improves the diagnostic specificity for iron deficiency anaemia & also provides a reliable work up for microcytic hypochromic anaemia.

Tests	Iron Deficiency anaemia	Anaemia of Chronic disease	Iron overload	Hemoglobinopathy (Especially Trait)
Serum Iron	Decreased	Decreased	Increased	Normal
Serum Total Iron Binding Capacity	Increased	Decreased or Normal	Increased or Normal	Normal
% Transferrin Saturation	Decreased	Decreased or Normal	Increased or Normal	Normal
Serum Ferritin	Decreased	Increased	Increased or Normal	Normal
Serum Soluble Transferrin receptor	Increased	Normal	Decreased	Normal
Serum Hepcidin	Normal	Increased	Normal	Normal

Associated Tests :
1. Serum Soluble Transferrin receptor
2. Serum Hepcidin

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Healthology Labs
Processed at an NABL accredited lab
Add: Healthology Labs, First Floor, B-23 H. K. Arcade, KSSIDC, Industrial Area, Mahadevapura, Bengluru-560 0048 | Email : reports@healthologylabs.com | Phone : 080-46807733

SAMPLE REPORT

Reference: SELF
Sample Collected At:
PROMISO HEALTH PRIVATE LIMITED - CREDIT
B-23, H K ARCADE 1ST, FLOOR 1B, MAHADEAVAPURA POST, I T I INDUSTRIAL AREA,
Sample Processed At: Metropolis Healthcare Ltd E-21, B1 Mohan Co-op Ind Estate New Delhi-110044

Investigation	Observed Value	Unit	Biological Reference Interval
Ferritin (Serum,CMIA)	96.9	ng/mL	13-150

Interpretation :

1. Increased ferritin is seen in iron overload as in multiple blood transfusions, hemochromatosis and anemia of chronic disorders .
2. Decreased ferritin levels are seen in iron deficiency anemia, early stage before iron deficiency manifests as anemia.
3. Increased ferritin is also seen in liver disease,alcoholism, inflammatory conditions, leukemia, Hodgkins disease and some malignancies.
4. Levels of ferritin are used for monitoring of iron levels during pregnancy, dialysis and during iron therapy

Abbreviation :
CMIA: Chemiluminescence Microparticle Immunoassay

-- End of Report --

MC-2676

Tests marked with NABL symbol are accredited by NABL vide Certificate no MC-2676; Validity till 04-05-2024

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